Regional Trends in Hospitals in the United States

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Abstract

Regional disparities in healthcare access and health outcomes are well-documented in the United States. Rural localities face worse health outcomes and greater challenges to accessing care than urban localities, particularly in the South. This study examines national and regional trends in hospital establishments (openings and closings), employment, size (employees per establishment), and regional concentration over three decades. We document an era of declining hospital counts and increasing employment within the industry, highlighting how the trends have differed across regions and rurality. Finally, we consider how these trends relate to the current policy environment in the United States.

Keyword: hospitals, rural healthcare, consolidation

JEL Classification: I10, I14, R12

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Introduction

Regional disparities in health outcomes remain a major challenge for public health and for community economic development across the United States. Rural localities consistently face worse health outcomes than their urban counterparts, and this is highlighted by persistently higher mortality rates in rural America, particularly in the South (Miller & Vasan, 2021). A key underlying issue is access to care. Recent expansions of telehealth following the COVID-19 pandemic have led to some progress towards greater access for rural communities, but obstacles remain. As such, hospitals continue to be an important anchor for both healthcare as well as local economic activity and employment. 16.7% of GDP in the United States is in the healthcare system (World Bank, 2026) and hospitals play a significant role within this industry.

This paper investigates how the landscape has changed in recent decades for American hospitals in terms of number of establishments, number of employees, number of employees per establishment (size), and regional concentration. We leverage data from the Census Bureau's Longitudinal Business Database (LBD) from 1987 to 2019. Because publicly available datasets often suppress localized data in low-population areas to protect confidentiality, it is difficult to get a granular look at the hospital industry, however, the restricted-access microdata from the LBD provides geographically precise establishment-level information that allows us to accomplish this. Using this three-decade panel, we highlight national and regional trends and document pronounced declines in hospital counts and increases in employment, which suggests a shift toward industrial consolidation. Importantly, we demonstrate how these trends have developed unevenly across space, disproportionately affecting southern and rural counties. Finally, we discuss how these trends fit within the current policy environment, considering how

emerging digital infrastructure initiatives and federal funding changes may alter the trajectory of community health and regional economic resilience.

Health, Access to Hospitals, and Community Economic Development

Regional disparities in health outcomes in the United States are well documented. Rural localities fare worse than urban localities on many measures. For example, rural counties have worse measures of social determinants of health than urban counties, and current trends suggest that the gap is widening (Weeks et al., 2023). Additionally, people living in rural areas face higher morbidity and mortality rates, particularly in the South, especially people of color (Miller & Vasan, 2021). Rural Americans face higher mortality partly because of greater risk for cardiovascular disease, myocardial infarction, heart failure, and stroke which also tend to be worst in the South (Sekkarie et al., 2025; Loccoh et al., 2022; Siddiqi et al., 2022). Finally, rural residents face worse cancer outcomes due to higher rates of risk factors like smoking and obesity as well as lack of access to innovative cancer prevention, diagnosis, and treatment (Bhatia et al., 2022).

One of the underlying drivers of regional disparities in health outcomes is regional disparities in healthcare access. In semi-structured interviews with primary care physicians in western Pennsylvania, Maganty (2023) and coauthors identified three themes relating to barriers to rural health care: cost and insurance, geographic dispersion, and provider shortage/burnout. Recent growth in telemedicine (Batool & Lopez, 2023) represents some progress toward addressing the first two barriers, but those who are uninsured or who live in communities with limited broadband coverage still face obstacles in obtaining care (Zhang et al., 2024).

Underlying both healthcare access and health outcomes is the importance of hospitals. Hospitals can be a healthcare anchor for local communities and proximity to hospitals is associated with decreased mortality from heart attacks and unintentional injuries. Proximity is also associated with increased perception of ease of healthcare access among older adults (Buchmueller et al., 2006). In addition to providing healthcare services, hospitals play a key role in local economies, particularly as an employment center for the healthcare industry. Evidence from hospital closures suggests that counties that experience hospital closures often see significant reductions in healthcare employment (Alexander & Richards, 2023; Chatterjee et al., 2022; Malone et al., 2022) as well as decreases in per capita income and increases in overall unemployment (Holmes et al., 2006). Moreover, hospital closures can significantly increase emergency department visits and admissions for hospitals in neighboring areas, which may be understaffed (Ramedani et al., 2022).

National Trends in Hospitals: A Story of Consolidation?

We use establishment-level data from the Census Bureau's Longitudinal Business Database (LBD) from 1987 to 2019 to conduct our analysis. These data are uniquely positioned to investigate hospitals for several reasons. First, using the LBD allows us to provide estimates with limited noise-injection or suppression. Publicly available data often does not disclose results at more precise granularities, which can cause measurement error in sparsely populated areas or when looking at a specific industry (in this case, NAICS 622). Second, the almost three-decade long analysis period allows us to characterize much of the recent history of the hospital industry not only for the nation but also across the urban-rural hierarchy and regions. For our analysis, we define a hospital as an establishment with at least fifty employees. We chose this employment

cutoff to illustrate how hospitals and health systems at a certain scale are changing rather than including hospitals with few employees.

Figure 1 presents the number of hospitals with at least fifty employees from 1987 to 2019. Hospitals of this size declined steadily between the late 1980s and early 2010s: establishment counts peaked in 1988 with over 6,400 hospitals and then fell more than 20% to about 5,040 in 2010. There was a slight increase in the number of hospitals immediately following the Great Recession until the end of our study period. This increase might be due to health care spending provisions in the American Recovery and Reinvestment Act of 2009, which included offering financial incentives to physicians and hospitals, funding for the construction and renovation of healthcare establishments, and continuing health insurance coverage for unemployed workers (Steinbrook, 2009). Overall, the number of hospitals fell sharply over the course of the study period.

This decline is in contrast with the growing share of GDP devoted to health expenditures previously mentioned in the introduction. Another way to measure business activity within an industry is employment. Figure 2 shows that the number of hospital employees increased over the period from just under four million in 1987 to almost 5.6 million in 2019. This increase of more than 40%, paired with the 20% decline in hospital counts presented in Figure 1, implies that the average size of hospitals has increased by 75% in the past thirty years. This highlights two key features of the industry. First, hospitals are getting bigger. Hospital administrators may find that growth allows them to take advantage of increasing internal economies of scale (i.e., declining average costs as hospitals get larger). Second, this industry-level evidence suggests hospitals have consolidated over this period. Although we do not directly observe mergers and acquisitions, studies have found that health systems have merged with or acquired physician

practices of varying sizes to grow (Government Accountability Office, 2025). Through this behavior, the number of hospitals has fallen.

As some hospitals close while others get bigger, a concern is whether this translates into reduced access to healthcare. Indeed, a feature of regional economic activity is that it is distributed unequally across space (Allen and Arkolakis, 2025), which could lead to some regions completely losing access. To capture this, we bin counties into three categories: counties with no hospitals, counties with one hospital, and counties with two or more hospitals. If the percent of counties with no hospitals rises during this period, populations may lose geographic access to healthcare. If much of the loss of establishments occurs in the counties-with-2-or-more category, the decline in hospital counts may be an outcome of local competition or consolidation.

Table 1 presents how these groups changed from 1987 to 2019. Across all three decades, the "one hospital" category holds the largest share of counties. In the decade starting in 1987, 46.1% of counties had one hospital. By the decade starting in 2009, this share increased to 49.8%. This increase is paired with a 7.2 percentage point decrease in the percent of counties with two or more hospitals, showing that local consolidation and competition occurred within counties. However, declining hospital counts did lead many counties to lose all their hospitals: the share of counties in the United States with no hospital increased by 3.6 percentage points over the period. Consolidation and competition has potentially led to geographic inaccessibility to healthcare across the nation.

The South Leads in Regional Trends

The national trends of declining hospital counts, increasing average size of hospitals in terms of employment, and increased geographic inaccessibility to healthcare might not

adequately capture the problems different regions face. We divide our national sample into the Census Bureau's four Census Regions to explore these regional differences.[2] Figure 3 presents the number of hospitals in each of the four regions over the course of the study period. Throughout the period, the South had the most hospitals followed by the Midwest, the West, and the Northeast. All four regions faced a decline in establishment counts, but the South's decline was the largest in magnitude, falling by about four hundred hospitals.

Figure 4 illustrates the number of people employed in these hospitals by region. Like Figure 3, the South has the most employees among the four regions followed by the Midwest. However, the Northeast has more employees than the West. The South and the West have the largest gains in hospital employment over the study period, while counts remained flat in the Midwest and the Northeast. Overall, the South has the biggest healthcare sector in both establishments and employment among the regions.

Taking Figures 3 and 4 together, we see that average hospital size is increasing across the period for all regions. The South leads with the largest loss in establishments and largest gains in employment over the period. These trends coincide with the South's rapid population growth since the 1950s due to the increased productivity of its regional economy and increases in housing supply (Glaeser and Tobio, 2008). Even so, the South's 16.7 percent decrease in establishments, 57.9 percent increases in employment, and 89.5 percent increase in average size rivals changes in the other regions. The Northeast faced the largest decline in establishments in

[2] The four regions are the Northeast (Connecticut, Maine, Massachusetts, New Hampshire, New Jersey, New York, Pennsylvania, Rhode Island, and Vermont); the Midwest (Illinois, Indiana, Iowa, Kansas, Michigan, Minnesota, Missouri, Nebraska, North Dakota, Ohio, South Dakota, and Wisconsin); the South (Alabama, Arkansas, Delaware, District of Columbia, Florida, Georgia, Kentucky, Louisiana, Maryland, Mississippi, North Carolina, Oklahoma, South Carolina, Tennessee, Texas, Virginia, and West Virginia); and the West (Alaska, Arizona, California, Colorado, Hawaii, Idaho, Montana, Nevada, New Mexico, Oregon, Utah, Washington, and Wyoming).

percentage terms (24.2%), the West faced the largest increase in employment in percentage terms (60.4%), and the West faced the second largest increase in average hospital employment size (84.4%). We focus on the South and how it compares to other regions for the rest of this section due to its rapidly changing healthcare industry and its standing as a region that suffers from poor health outcomes.

As the South leads in increases in hospital size across regions, a natural question is whether it also has a greater share of counties with no hospital. Table 2 compares these percentages for the South against the rest of the United States. Most counties in both the South (47.8%) and the rest of the United States (43.3%) contain one hospital in the decade starting in 1987. These percents then converge to about 50.4% in the decade starting in 2009, meaning that around half of all counties in both regions have one hospital. Both regions face similar trends in the other two categories: the percent of counties with two or more hospitals is declining, while the percent of those with zero hospitals is increasing. The South has a consistently higher share of counties with zero hospitals: in the decade starting in 2009, 26.9% of counties in the South do not have hospitals compared to only 19.7% of counties in the rest of the United States. While differences in county size might account for some of this difference, it should be noted that the percentage of counties without a hospital increased more in the South (20.6%) than in the rest of the United States (16.6%) between 1987 and 2019. This suggests that, despite having the most hospitals and employees, regional inaccessibility to healthcare may be growing in the South relative to the rest of the United States. Again, it should be noted that county size, density, and other attributes may vary in the South compared to other regions, so more research is needed to determine the exact degree to which inaccessibility may be worsening.

Differences in hospital ownership in the South might contribute to geographic inaccessibility. Nonprofit hospitals may exist to improve equity in access to healthcare across counties and income groups, while for-profit firms may seek only to maximize profit (Bhattacharya et al, 2014). Table 3 presents the share of counties for the South and the rest of the United States that have only nonprofit, only for-profit, or both types of hospitals within their borders. Over all three decades, the South has a drastically larger share of counties with only for-profit hospitals. For example, 46.6% of counties in the South are home to only for-profit hospitals compared to 32.4% in the rest of the United States in the decade starting in 2009. Rather than coinciding with major differences in counties that have both types of hospitals, this difference corresponds to a significant difference in counties that have only nonprofit hospitals. Indeed, the South has about 12.2 percentage points less counties with only nonprofits (41.1%) than the rest of the United States (53.3%) in the decade starting in 2009. While the share of counties with nonprofit hospitals has been increasing and for-profit has been decreasing over time for both regions, this difference in ownership overlaps with the South's consistently larger healthcare industry with less geographic access.

Hospital Churn and Geographic Accessibility Vary Across Urban-Rural Hierarchy

Hospitals are important for the health of rural residents and can serve as anchor institutions in local economies (Van Leuven et al., 2026). To capture the evolution of declines in hospital counts and geographic inaccessibility in rural areas, we use the U.S. Office of Management and Budget delineations of metropolitan, micropolitan, and rural (not metropolitan or micropolitan). We also identify when specific hospitals open and close using the establishment-level data. Because hospitals sometimes change ownership or classification, we

took steps to systematically verify openings and closings as valid and persistent. We define a last hospital closing as a closing that leaves a county with no hospital.

To what extent does geographic inaccessibility change across rurality between the South and the rest of the United States? Table 4 presents results akin to Table 1 and Table 2. Several features stand out. First, a higher percentage of rural counties have zero hospitals than micropolitan and metropolitan counties in both the South (39.2% in the decade starting in 2009) and the rest of the United States (30.1% in the decade starting in 2009). At such an elevated level, it makes sense that last hospital closures are fewer in rural areas (Table 5). Second, the South leads the rest of the United States with zero hospitals in rural and metropolitan areas by approximately four percentage points to ten percentage points in any given decade. Third, more than half of rural and micropolitan counties in both the South and the rest of the United States have one hospital.

Table 5 shows the number of hospital openings, closings, and last closings across the three rurality designations from 1988 to 2017. As one might expect from increased demand for healthcare due to population size and density, urban areas exhibit the most hospital openings and closings (i.e., churn) in magnitude, followed by micropolitan and rural counties. Across all rurality designations, the total number of hospitals that open and close falls, supporting the notion that consolidation and internal economies of scale might stifle competition through higher entry costs. Last hospital closings remained consistent in micropolitan and metropolitan areas but have decreased in rural areas. This trend might be due to the higher number of counties without a hospital throughout the period, a phenomenon discussed later in this section.

Table 6 presents comparisons in hospital churn across rurality for the South and the rest of the United States. Last hospital closures have mostly been in rural areas, especially in the

South, and in micropolitan areas in the South. This is interesting in the context of the South also seeing the largest gains in hospital employment. In addition to representing a spatial reorganization of hospital location and concentration, this might also hint at an increase in migration and/or commuting for hospital employees and other healthcare workers.

As for trends across rural designations, the most prominent and concerning change occurs in the share of rural and metropolitan counties with zero hospitals. The share of rural counties with zero hospitals grew by seven percentage points in the South and two percentage points in the rest of the United States. Similarly, the share of metropolitan counties with zero hospitals increased by nine percentage points in the South and eight percentage points in the rest of The United States. Metropolitan counties that lose their hospital might be part of a larger labor market area, so this loss of geographic accessibility might not imply a loss of healthcare. For example, the counties in and surrounding the New York City metropolitan area might lose a hospital, but residents will still be able to commute to a nearby hospital. Rural counties, however, might suffer drastically from this loss as distance to the nearest hospital is longer and population density is sparser.

Emerging Policy Issues for Local Hospitals and Healthcare Access

Will growing geographic inaccessibility to healthcare continue in the United States, particularly in the South and in rural areas? We identify two important national developments that might affect these trajectories: (1) improved deployment and adoption of broadband (i.e., high-speed internet) and telehealth technologies through the Broadband Equity Access and Deployment (BEAD) program; and (2) changes to Medicaid and the Affordable Care Act (ACA) due to the One Big Beautiful Bill Act (OBBBA). While there are nuances to how these national

developments might affect hospitals, we conjecture that they will exacerbate the decades-long trends towards consolidation and loss of geographic access.

The United States was made acutely aware of the importance of reliable broadband for work, leisure, and health during the COVID-19 pandemic. While the federal government has invested in many rural broadband infrastructure projects in the past several decades (see Mack et al, 2024), the BEAD program is the newest endeavor, aiming to provide broadband (100Mbps download / 20Mbps upload) to every American by 2030 (Keene et al, 2024). This initiative coincides with improvements in telehealth implementation, ranging from improved technologies to insurance reimbursement parity between telehealth and in-person services. If telehealth provides services that are easily substitutable with in-person visits, rural patients may choose to virtually visit urban hospitals that provide higher quality services instead of lower quality local ones for nonemergency healthcare. Under this scenario, profit margins might fall for financially distressed hospitals, leading to shrinking establishments and closures. Whether improved virtual access to quality healthcare compensates for the loss of geographic access for patients would be an important line of future research.

The Medicaid program provides free medical care to low-income adults and children, the elderly, and the disabled. It is the largest means-tested transfer program in the United States (Moffitt, 2016). Because this program lowers the cost of healthcare for the patient, patients theoretically demand more medical care when covered. Studies have shown that favorable changes in Medicaid fees and reimbursements lead providers to admit more low-income patients, likely improving profit-margins (see Buchmueller et al, 2016). The OBBBA cuts Medicaid funding by approximately $886.8 billion over the next ten years, leaving an estimated 7.5 million people without health insurance (CBO, 2025). For hospitals with many Medicaid patients, this

loss in insurance might profoundly affect their bottom lines, potentially leading to more closures. As a result, OBBBA might aggravate consolidation and loss of geographic accessibility.

Where do we go from here?

This assessment of the hospital industry across three decades provides several important conclusions. First, the total number of hospitals has declined over time while the total number of hospital employees has increased significantly. Taken together, these trends suggest an era of hospital consolidation in which Americans are left with fewer, albeit larger, in terms of employment, hospitals. Second, trends in the hospital industry have been uneven across regions and rurality. While the South had the largest absolute gains in hospital employment, it also had the largest loss of establishments. By 2019, over a quarter of southern counties lacked a single hospital and the share of rural counties nationwide without a hospital had increased significantly since 1987.

Looking forward, these long-term trends are poised to collide with shifting federal policy priorities. While emerging digital infrastructure initiatives such as BEAD can expand telehealth access, such initiatives may also siphon revenue away from financially distressed rural hospitals, accelerating the rate of closures. More research is needed to determine whether telehealth is a suitable substitute for in-person healthcare. Imminent fiscal contractions such as those projected under OBBBA threaten to erode the Medicaid safety net, further undermining the financial viability of rural hospitals. Rural communities who rely heavily on hospitals, especially financially strained hospitals, for economic activity must evaluate potential pathways for economic development in the event of closure. Federal and state lawmakers should consider not

only national trends but also the regional nuances in the hospital and healthcare industry to design policy frameworks that address the disparate distribution of care.

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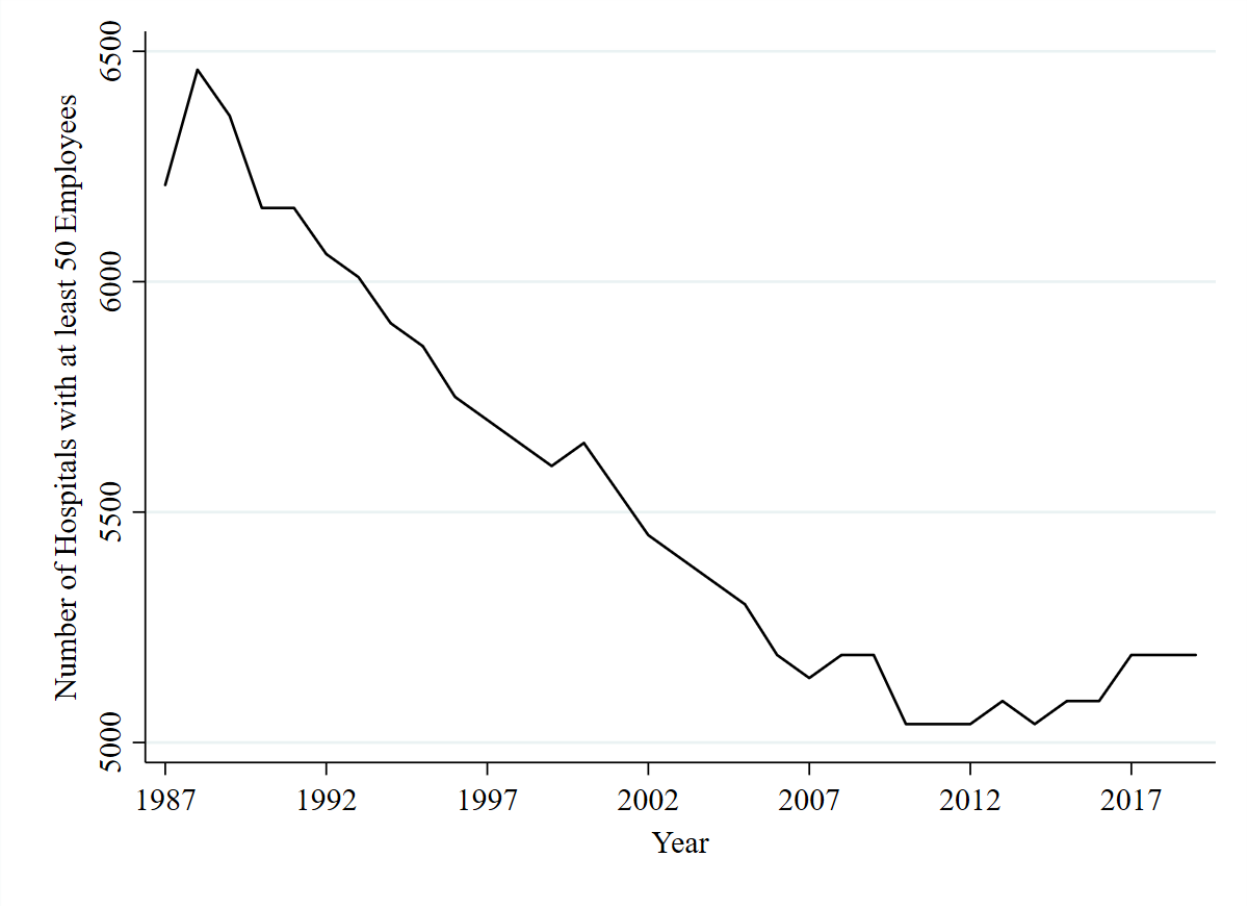
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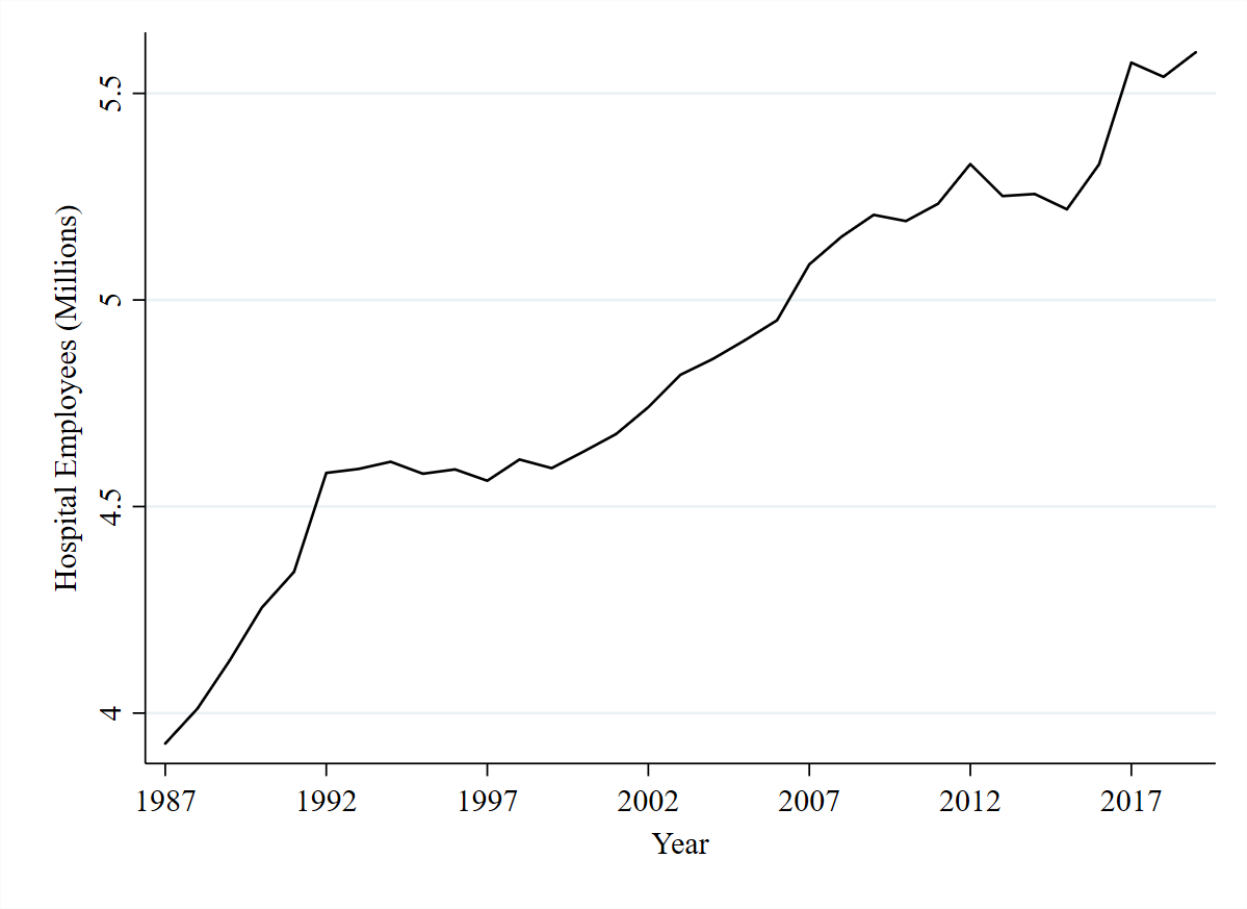
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Figure 1: Number of Hospitals with at Least 50 Employees in the United States, 1987-2019



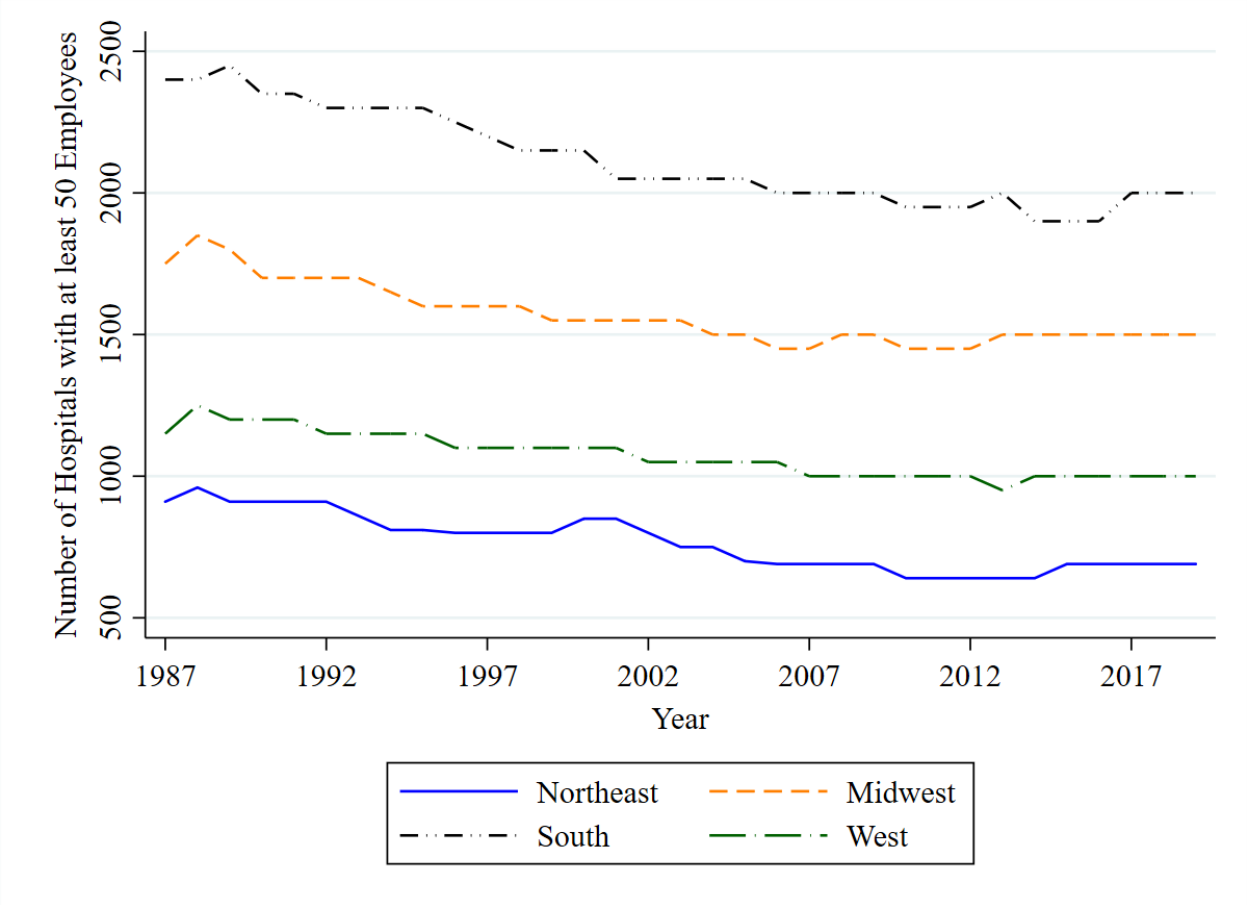
Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This figure plots the number of hospitals nationwide with at least fifty employees by year from 1987 through 2019. The figure shows that the number of hospitals declined significantly over this period.

Figure 2: Number of Employees in Hospitals with at Least 50 Employees in the United States, 1987-2019



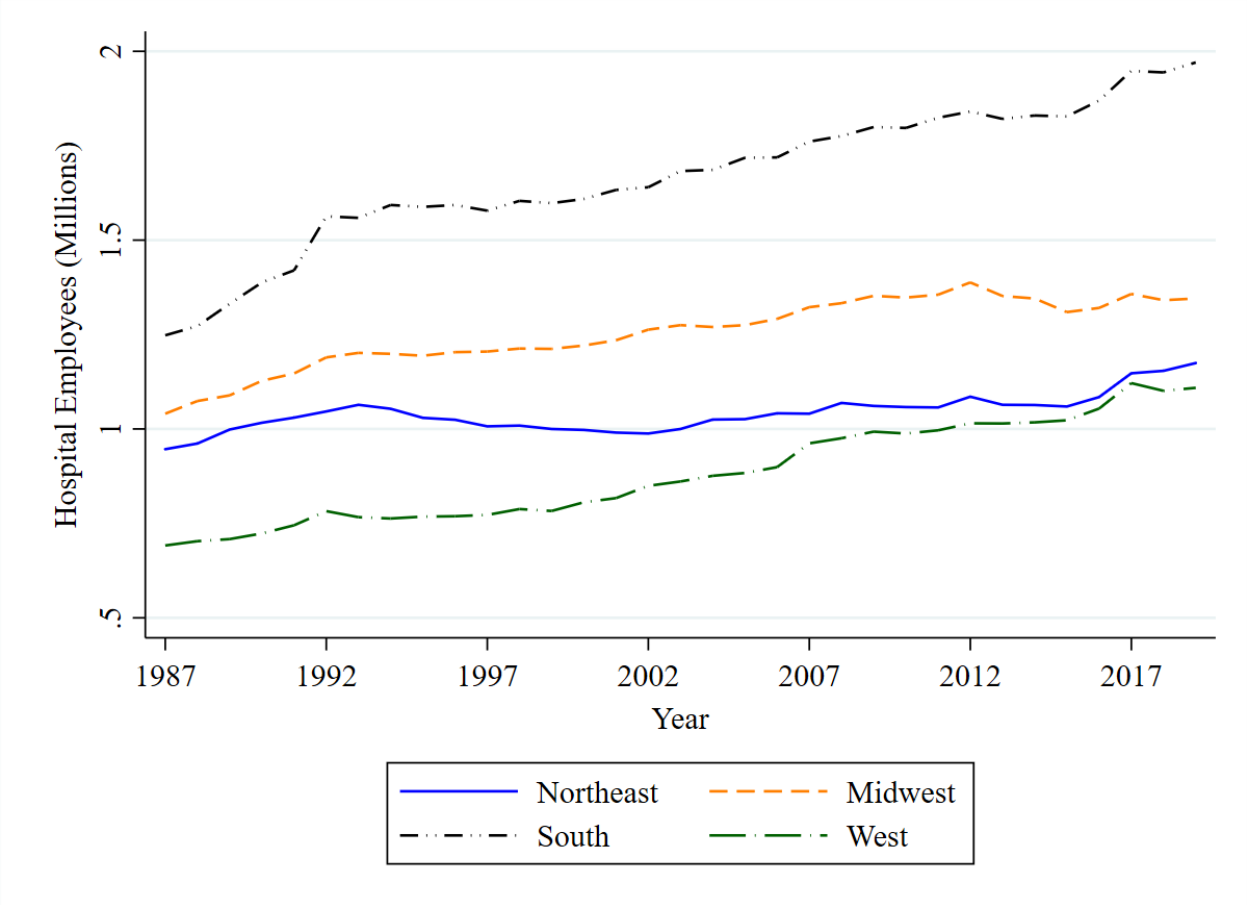
Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This figure plots the national total number of hospital employees in millions working in hospitals with at least fifty employees for each year from 1987 through 2019. The figure shows that the overall number of hospital employees has increased significantly.

Figure 3: Number of Hospitals with at Least 50 Employees in the United States by Region, 1987-2019



Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This figure plots the number of hospitals with at least fifty employees by Census region for each year from 1987 through 2019. The figure shows that the number of hospitals declined in each region, with the steepest decline in the South.

Figure 4: Number of Employees in Hospitals with at Least 50 Employees in the United States by Region, 1987-2019



Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This figure plots the total number of hospital employees in millions working in hospitals with at least fifty employees by region for each year from 1987 through 2019. The figure shows that the number of hospital employees increased in each region, with the steepest increases in the South and in the West.

Table 1: Hospital Concentration in U.S. Counties, 1987-2019

	Percentage of Counties		
Time Period	0 Hospitals	1 Hospital	2 or More Hospitals
1987 to 1997	20.3%	46.1%	33.6%
1998 to 2008	23.4%	49.3%	27.4%
2009 to 2019	23.9%	49.8%	26.4%

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes the percentage of counties nationwide with zero, one, or two or more hospitals by decade. The table shows that the proportion of counties with two or more hospitals decreased while the proportion of counties with zero or one hospital increased.

Table 2: Hospital Concentration in U.S. Counties in the South vs Rest of United States, 1987-2019

	Percentage of Counties					
	0 Hospitals		1 Hospital		2 or More Hospitals	
Period	South	Rest of U.S.	South	Rest of U.S.	South	Rest of U.S.
1987 to 1997	22.3%	16.9%	47.8%	43.3%	29.9%	39.8%
1998 to 2008	28.3%	18.6%	50.9%	45.6%	20.8%	35.8%
2009 to 2019	26.9%	19.7%	50.5%	50.3%	22.5%	30.0%

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes the percentage of counties in the South and in the rest of the U.S. with zero, one, or two or more hospitals by decade. The table shows that the proportion of counties with no hospital increased more in the South than in the rest of the country, the proportion of counties with one hospital increased more in the rest of the U.S. than in the South, and the proportion of counties with two or more hospitals decreased significantly, both in the South and the rest of the country.

Table 3: Hospital Ownership in U.S. Counties in the South vs Rest of United States, 1987-2019

	Percentage of Counties					
	Nonprofit Hospital(s)		For Profit Hospital(s)		Both	
Period	South	Rest of U.S.	South	Rest of U.S.	South	Rest of U.S.
1987 to 1997	23.9%	32.6%	65.0%	49.9%	11.1%	17.5%
1998 to 2008	38.5%	46.3%	47.9%	39.4%	13.6%	14.3%
2009 to 2019	41.1%	53.3%	46.6%	32.4%	12.4%	14.4%

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes the percentage of counties in the South and in the rest of the U.S. with a nonprofit hospital, a for-

profit hospital, or both types by decade. The table shows that the proportion of counties with a for-profit hospital decreased significantly nationwide.

Table 4: Hospital Concentration in U.S. Counties in the South vs Rest of United States by Rurality, 1987-2019

	0 Hospitals (% of Counties)						1 Hospital (% of Counties)						2 or More Hospitals (% of Counties)					
Period	South			Rest of U.S.			South			Rest of U.S.			South			Rest of U.S.		
	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro
'87 - '97	32.6	14.8	15.4	28.0	10.1	6.9	54.0	50.6	31.3	54.4	48.9	18.4	13.4	34.6	53.2	17.7	41.0	74.6
'98 - '08	36.9	16.5	19.5	29.6	9.5	11.8	53.6	58.6	34.4	56.9	54.0	20.4	9.5	24.9	46.1	13.5	36.5	67.7
'09 - '19	39.2	13.2	24.8	30.1	7.6	14.3	54.2	66.7	37.6	57.7	60.2	25.0	6.6	20.0	37.6	12.1	32.2	60.7

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes the percentage of counties with zero, one, or two or more hospitals in the South and in the rest of the U.S. by rurality and by decade. The table shows that proportion of counties with no hospitals is higher in the South and in rural areas.

Table 5: Hospital Openings and Closings in U.S. Counties by Rurality, 1988-2017

	Rural			Micropolitan			Metropolitan		
Decade	Hospital Openings	Hospital Closings	Last Hospital Closings	Hospital Openings	Hospital Closings	Last Hospital Closings	Hospital Openings	Hospital Closings	Last Hospital Closings
1988-1997	327	429	105	305	413	56	1117	1315	31
1998-2007	202	221	63	212	223	38	781	963	25
2008-2017	96	95	41	128	164	47	573	463	32

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes hospital openings, closings, and last closings (a closing that results in a county having no hospital) by rurality and by decade. The table shows that hospital activity (opening and closing) is greater in metropolitan areas but that last hospital closings have been more common in rural counties.

Table 6: Hospital Openings and Closings in U.S. Counties by Rurality, South vs Rest of United States, 1987-2019

	Hospital Openings						Hospital Closings						Last Hospital Closings					
	South			Rest of U.S.			South			Rest of U.S.			South			Rest of U.S.		
Period	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro	Rural	Micro	Metro
1987 to 1997	187	200	518	140	105	603	242	259	586	181	159	728	65	41	22	41	17	11
1998 to 2008	117	106	353	85	105	426	121	114	398	98	104	561	38	22	15	28	14	11
2009 to 2019	53	77	256	43	54	312	61	98	199	35	69	263	24	33	19	13	9	6

Note: FSRDC Project Number 2552. (CBDRB-FY25-P2552-R11813) This table summarizes hospital openings, closings, and last closings for the South and for the rest of the U.S. by rurality and by decade. The table shows that hospital activity is greater in metropolitan counties and that last closings have been more common both in rural counties and in southern counties.